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Submitted via the Federal eRulemaking Portal: <http://www.regulations.gov>

Robert F. Kennedy, Jr.
Secretary
Dr. Thomas Keane
Assistant Secretary for Technology Policy and National Coordinator for Health Information Technology
Department of Health and Human Services
Attention: Request for Information: HHS Health Sector AI RFI
Mary E. Switzer Building, Mail Stop: 7033A
330 C Street SW
Washington, DC 20201

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care/HHS Health Sector AI RFI [RIN 0955-AA13]

Dear Secretary Kennedy and Dr. Keane:

The College of Healthcare Information Management Executives (CHIME) appreciates the opportunity to comment on the Department of Health and Human Services (HHS) Office of the Deputy Secretary and Assistant Secretary for Technology Policy and Office of the National Coordinator for Health Information Technology (ASTP/ONC) request for information (RFI) on Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care, as published in the *Federal Register* on December 23, 2025 (Vol. 90, No. 244).

Background

CHIME is an executive organization dedicated to serving chief information officers (CIOs), chief medical information officers (CMIOs), chief nursing information officers (CNIOs), chief innovation officers (CIOs), chief digital officers (CDOs) and other senior healthcare IT leaders. With thousands of individual members, and hundreds of CHIME Foundation healthcare IT business and professional service firm members, CHIME and its communities provide a highly interactive, trusted environment enabling senior professional and industry leaders to collaborate, exchange best practices, address professional development needs, and advocate for the effective use of information management to improve the health and care in the communities they serve.

Key Recommendations and Takeaways

Our members – hospital and healthcare system CIOs, CISOs, and digital health leaders – are early adopters and stewards of AI technologies and are acutely aware of both their promise and their risks. We welcome HHS's recognition that regulatory frameworks, reimbursement policies, and research investments must evolve in tandem to ensure AI tools are safe, effective, and deployable at scale. CHIME strongly believes that we must responsibly advance AI adoption,

improve patient outcomes, reduce clinician burden, and lower healthcare costs, while maintaining the trust and confidence of patients and providers.

AI is no longer a distant concept. It is reshaping the very foundation of healthcare delivery. From enhancing diagnostic accuracy to streamlining administrative workflows, AI offers unprecedented opportunities to improve patient outcomes, reduce clinician burden, and foster innovation across the care continuum. Yet, with these opportunities come profound responsibilities.

Healthcare providers stand at the intersection of technology and trust. As stewards of patient safety and privacy, they must navigate a rapidly evolving landscape where algorithms influence clinical decisions and data flows beyond traditional boundaries. The [CHIME Principles for Responsible AI](#) provide a comprehensive approach describing both challenges and potential solutions to help nurture responsible AI adoption. They serve as a guide to understanding and implementing AI responsibly anchored in principles of transparency, fairness, and patient-centered care.

Core Pillars of CHIME's AI Principles

- **Protecting Patients and Their Data:** Outlines expectations for robust cybersecurity practices, streamlined privacy laws, clear patient consent policies, and standardized security requirements for third-party technologies.
- **Fostering Patient-Centered Care:** Emphasizes the importance of human oversight, adoption of voluntary frameworks and standards, risk-based monitoring, and ongoing transparency regarding AI's role in clinical decision-making.
- **Empowering Providers:** Highlights the need to equip providers, especially small and under-resourced organizations, with education, resources, and support while promoting transparency, innovation, and equitable access to AI-enabled tools.

As HHS recognizes, AI is inherently data-driven and dependent on robust interoperability, governance, and safeguards to ensure appropriate use, privacy, and security. The Department's RFI is timely and necessary as providers navigate a rapidly evolving AI ecosystem in which clinical utility, regulatory clarity, reimbursement pathways, and liability frameworks remain fragmented and uncertain. We offer the following comments to highlight practical considerations from frontline providers and to recommend policy approaches that balance innovation with patient safety, provider accountability, and sustainable implementation.

Detailed Input & Recommendations

In this RFI, HHS is seeking concrete, experience-based feedback from those building, buying, evaluating, using, and receiving care from AI tools that are part of clinical care as well as from those who wish to do so but face barriers. CHIME's feedback addresses the questions outlined in this RFI, which seeks public feedback on the actions the Department can take to establish a forward-leaning, industry-supportive, and secure approach to accelerate the adoption and use of AI as part of clinical care.

AI holds significant promise to enhance clinical decision-making, streamline workflows, and improve patient access and outcomes. At the same time, successful and sustainable AI adoption requires clear, risk-based regulatory pathways, interoperable data infrastructure, and policies that foster innovation while safeguarding patient privacy and security. HHS's recognition

that federal regulatory, reimbursement, and research policies must be better aligned to support responsible AI integration is both timely and necessary.

CHIME's input seeks to provide practical, provider-informed recommendations to help the Department advance a flexible, innovation-enabling policy framework that supports real-world clinical implementation without imposing duplicative or prescriptive requirements that could impede adoption or increase provider burden. AI is not a replacement for human judgment; it is a tool to augment clinical expertise. By embracing innovation while safeguarding trust, HHS can ensure that AI fulfills its promise: improving health for all.

AI implementation in clinical care raises complex questions regarding data governance, interoperability, workforce readiness, liability, and a growing “digital divide.” HHS's focus on patient trust, data security, and interoperability reflects foundational prerequisites for scalable AI adoption and aligns with long-standing provider priorities. CHIME members wish to emphasize the need for clear, risk-based policies, alignment with existing health IT frameworks, and pragmatic solutions that reflect the realities of healthcare system operations.

CHIME members are actively deploying and evaluating AI-enabled tools in clinical, administrative, and operational settings. However – broadly, private-sector AI innovation and clinical adoption for healthcare providers are slowed by financial, technical and data constraints, regulatory and market-structure misalignment, and organizational challenges in implementation and scale.

Peer-reviewed evidence indicates that AI-based clinical decision support tools embedded within electronic health records (EHRs) are widely deployed, in part because certain tools have historically fallen outside FDA oversight and because activation costs are perceived as modest; however, clinicians and health systems continue to raise concerns regarding accuracy, clinical value, and utility.¹ The study further found that, “Outside [of] medical imaging and EHR-embedded applications, dissemination of clinical AI tools has been slower. The primary barrier is likely that healthcare systems responsible for buying these tools do not believe they offer enough value. The costs of not just licensing a tool but also of ensuring adequate training, digital infrastructure, maintenance, and monitoring can be considerable. There is no reimbursement for most AI tool use, and even in select cases where insurers provide reimbursement, it does not offset the full costs.”¹

For those under-resourced providers attempting to adopt AI, some simply lack the purchasing power to afford it, including those solutions available through their EHR. AI uptake is also occurring at a higher rate among larger providers – while smaller, lesser-resourced providers struggle to keep pace with long-standing issues like solvency and reimbursement.

A study [published in JAMIA in May 2025](#) examining mid- to large-provider adoption of AI concluded that systems with greater financial and technical resources are more likely to adopt and scale AI technologies. The rationale for AI adoption cited most frequently among respondents was related to clinician burden. All survey respondents indicated successful adoption of ambient AI but outside of this use case, AI adoption was uneven among providers. The authors found that “Other select AI use cases such as imaging/radiology and clinical risk stratification are adopted by many, though successes are modest.”

¹ Angus DC, Khera R, Lieu T, et al. AI, Health, and Health Care Today and Tomorrow: The JAMA Summit Report on Artificial Intelligence. JAMA. 2025;334(18):1650–1664. doi:10.1001/jama.2025.18490

National Frameworks are Needed

CHIME continues to reiterate that the U.S. needs a comprehensive national data privacy law to better protect consumers' sensitive health information and inform consumers of how their data is being used without duplicating what is already required under HIPAA. Furthermore, CHIME members believe that a federal privacy law **and** a federal AI law that preempts state laws are both needed to reduce burden and improve efficiency.

The amount of health data being created using AI is staggering in size. Advanced models analyzing x-rays, magnetic resonance imaging (MRIs), clinical notes, and genomic data contribute massive amounts of unstructured data. This data is in addition to the data being created by smart devices that are continuously collecting health metrics. A single major hospital can generate upwards of 50 petabytes of data annually which translates to approximately 170 years of HD video footage according to the [World Economic Forum](#). The exponential growth of health data requires heightened responsibility from healthcare providers to safeguard its security, integrity, and responsible application.

The existing state patchwork of privacy laws has created an enormously complicated legal landscape for healthcare providers and other stakeholders. Varying sets of state laws present complexity, burden, and added costs to the healthcare system. Thus, the need for a national data privacy law has become increasingly important. Five years ago, there were five states with privacy laws enacted; today 20 states have privacy laws. As providers move to adopt AI and the types of tools they are adopting are more complex, the privacy challenges are only mounting. A federal law that preempts the myriad of state laws could bring greater efficiency to providers and reduce administrative burden and complexity.

In the absence of a federal law on AI, and interest in AI at an all-time high, states continue passing their own laws which have addressed mental health AI tools (i.e., chatbots), AI in insurance and claims, clinical decision support systems, patient data privacy and algorithmic transparency. Similar to the need for a federal privacy law that preempts state laws, a single federal AI law that preempts state laws is needed.

AI Regulation, Reimbursement, and Research & Development

Regulatory Clarity and Flexibility will Spur Innovation

CHIME respectfully submits that a modernized, streamlined regulatory framework is essential to unlocking the full innovative potential of AI in clinical care. While appropriate safeguards remain critical to protect patients and maintain public trust, overly prescriptive or duplicative requirements can inadvertently delay deployment, increase costs, and divert scarce resources from patient care, and innovative cybersecurity investments. To foster meaningful innovation, federal policy should emphasize regulatory clarity, and flexibility – creating an environment in which healthcare providers can responsibly test, refine, and scale AI solutions without unnecessary administrative friction. Thoughtful regulatory reform will not diminish accountability; rather, it will enable providers to innovate more rapidly and effectively in service of better outcomes, lower costs, and a more resilient healthcare system.

Navigating Patient Consent

Providers are using AI in a variety of ways. As AI becomes increasingly embedded in clinical workflows, healthcare providers face mounting pressure to modernize patient consent policies.

A central challenge lies in distinguishing between explainable AI, which attempts to explain how an algorithm arrived at its answer or output, and interpretable AI, which allows users to more intuitively understand how an AI-driven decision was reached. While both frameworks aim to enhance trust and transparency, they present different implications for consent. Explainable AI often requires complex, technical justification that may overwhelm or confuse patients, whereas interpretable AI lends itself to clearer communication but may be limited in scope.

As more providers adopt AI, they must determine how to handle informing the patients they treat about its use in their care. Determining how and when to inform patients, and to what degree it is being used is still an evolving area for providers. The integration of AI into clinical workflows – ranging from ambient listening tools (a voice recognition technology that captures a patient interaction with a clinician) to traditional clinical decision support systems to chatbots – raises difficult questions about what, when, and how to best disclose its use.

As AI tools such as clinical decision support systems, ambient listening technologies, and scheduling assistants become more prevalent in healthcare, it's increasingly important to clarify how patient consent applies under HIPAA's Treatment, Payment, and Healthcare Operations (TPO) framework – particularly since many of these tools may operate in the background or across multiple functions. For example, ambient listening used for clinical documentation may fall under 'treatment' or 'operations' depending on its use, while revenue cycle AI tools clearly support 'payment.' Clarifying these distinctions helps ensure that patients understand when their data can be used without explicit consent under TPO, and when additional authorization may be required.

In some organizations, consent is obtained at the beginning of the patient's care. The challenge lies in determining how much detail to disclose about AI and machine learning (ML) tools that can impact patient care. While some argue that disclosing every automation would overwhelm patients, others believe that complete transparency is crucial. To address this, some institutions have built an inventory of AI tools and share summaries with internal teams, with plans to make this information available to patients in the future. They use platforms, including their websites, to inform patients about AI usage rather than providing notifications for every instance. Surveys indicate that patients do not want to be notified every time AI is used. **This underscores the need to consider whether a single consent form should be used to inform patients about AI usage.**

Providers continue to evaluate how extensively AI should be integrated into their organizations, as they are cautious about increasing the discoverability of sensitive information and are carefully considering the best approach to ensure patient consent and data privacy. As AI continues to permeate the healthcare delivery landscape, one member reflected, "getting consent for each and every use of AI will be wildly impractical."

Balancing Innovation with Appropriate Oversight and Support for Providers

Current regulatory frameworks provide an important baseline for oversight of health information technology; however, the 21st Century Cures Act² includes exemptions that have led to inconsistencies in the regulatory treatment of third-party AI tools and applications. One CHIME member – a practicing emergency physician – emphasized that these inconsistencies can allow certain AI-enabled functionalities to evade FDA oversight, creating gaps in safety and effectiveness assurance as generative and agentic AI increasingly blur the line between

² Pub. L. No. 114-255, § 3060 (2016)

administrative and clinical use and strain traditional intended-use frameworks. HHS could further clarify and modernize these exemptions to ensure appropriate oversight of high-risk AI tools while preserving flexibility for low-risk innovation. For example, the January 2026 update to the U.S. Food and Drug Administration's (FDA) [General Wellness: Policy for Low Risk Devices](#) guidance demonstrates a thoughtful approach to delineate when certain digital health and software functions fall outside traditional medical device oversight based on intended use and risk. The guidance reiterates that software functions intended solely to maintain or encourage a healthy lifestyle and are unrelated to the diagnosis, cure, mitigation, prevention, or treatment of disease may fall outside the statutory device definition, while other low-risk products may be subject to enforcement discretion.

Given that FDA operates within HHS, CHIME encourages the Department to further promote greater cross-agency alignment and clarity regarding how risk-based oversight frameworks apply to AI-enabled technologies across the continuum of wellness and clinical care. Doing so would help ensure that regulatory boundaries are sufficiently clear for providers, support appropriate accountability for higher-risk third-party AI tools, and reduce ambiguity that can slow responsible and innovative adoption.

A recent peer-reviewed analysis published in *JAMA* highlights significant ambiguity in accountability for evaluating the health effects of AI tools in healthcare. The article observes that, while developers of AI tools subject to FDA review bear initial responsibility for evaluation, such assessments may not extend to real-world clinical outcomes. Developers of tools exempt from FDA clearance often design evaluations around product claims – such as subscriber engagement for direct-to-consumer applications or financial performance for revenue cycle management solutions – without systematically assessing potential health impacts. Healthcare delivery organizations (HDOs) may conduct independent evaluations, yet many lack sufficient funding or specialized expertise to do so comprehensively. Federal support through grants exists but is limited, leaving patients and communities – critical stakeholders whose insights are essential to understanding real-world effects – routinely excluded from these assessments.¹

Mitigating Risk and Liability for Providers

AI use in healthcare is mostly voluntary at present, but as evidence of its effectiveness grows, not using AI could be seen as falling below the standard of care, potentially exposing providers to malpractice claims. Conversely, when AI is used and an adverse outcome occurs, liability could involve the clinician, the health system, or the AI developer. Although relevant case law is currently limited, all parties must have strategies to address potential liability risk.¹ Healthcare providers and systems face well-established risks – including malpractice liability, reputational harm, regulatory or board sanctions, loss of medical licensure, credentialing restrictions, and financial exposure – whereas AI developers are generally far better positioned financially to withstand potential legal challenges.

Therefore, CHIME members believe that one of the most urgent policy needs is a clear and workable framework for patient consent and liability related to the use of generative AI in clinical care delivery. As generative AI tools become increasingly embedded across clinical, administrative, and operational workflows, providers anticipate that such tools will soon be ubiquitous and integral to routine care delivery.

It is neither feasible nor clinically appropriate for patients to opt out of all AI-enabled functions within modern health systems, and that highly granular disclosures for each individual AI use case would be operationally impractical and could undermine care delivery. At the same time,

the absence of clear federal guidance creates uncertainty regarding consent obligations and litigation risk.

Accordingly, CHIME recommends the establishment of a standardized, technology-neutral patient consent framework and a legal safe harbor for the use of generative AI in healthcare, analogous and aligned to existing informed consent and privacy frameworks (i.e., HIPAA), to reduce legal ambiguity, support responsible innovation, and protect both patients and providers. Such a framework could include model language, baseline disclosure principles, and liability protections for good-faith use of AI consistent with federal standards, thereby mitigating the risk of fragmented state-level requirements and opportunistic litigation while preserving patient trust and transparency.

Removing Cost Barriers

CHIME members report that, although AI-enabled tools are improving care delivery and patient engagement, current reimbursement structures do not adequately account for the associated costs. One member described deploying an AI-driven agent to proactively contact patients with hypertension and other chronic conditions to improve monitoring and adherence. While the organization anticipates downstream clinical and financial benefits from improved outcomes, it is currently absorbing the upfront and operational costs because no direct reimbursement pathway exists for these services.

Similarly, another member described comparable challenges during telehealth implementation, where the costs of equipment, infrastructure, and staffing facilitators exceeded available reimbursement, despite measurable improvements in access and quality. These experiences underscore a persistent misalignment between innovation that advances patient care and payment models that fail to recognize or support the associated investments. In the absence of sustainable reimbursement mechanisms, providers are effectively required to subsidize technologies that enhance quality, safety, and patient engagement – an approach that is not scalable over time.

One CHIME member – a regional safety-net/anchor system – shared that emerging technologies, including AI-enabled clinical tools, are evaluated through structured governance processes based on defined clinical use cases, business need, return on investment, and the level of autonomy and associated patient safety risk. A core principle of these governance frameworks is education and accountability – recognizing that, in the absence of comprehensive regulatory oversight, providers bear primary responsibility for determining whether a technology is safe, effective, and appropriate for clinical deployment.

To operationalize risk-based decision-making, this organization classifies AI and other emerging technologies into categories such as novel, platform, or commodity, with each category informing tailored acceptance criteria and risk-benefit thresholds. This tiered approach reflects the need for differentiated regulatory and oversight expectations based on maturity, autonomy, and clinical impact, and underscores the importance of aligning federal policy with provider-driven risk governance models.

CHIME members cite AI-enabled radiology tools as illustrative of the current reimbursement gap. These technologies are increasingly integrated into clinical workflows to perform preliminary computational image analysis, support clinical decision-making, and algorithmically prioritize radiologist worklists based on predicted acuity. Despite their measurable impact on

workflow optimization, timeliness of interpretation, and patient safety, existing payment methodologies do not clearly recognize or account for these technology-enabled services.

In practice, these systems rapidly analyze routine imaging studies upon acquisition, flag potential abnormalities, and elevate high-risk cases for expedited clinician review. One member indicated that this AI-driven triage can reduce turnaround times for routine films from approximately 20 hours to roughly one hour, materially improving clinical workflow efficiency and patient care timeliness. These tools are used as an adjunct to, rather than a replacement for, clinician review, with radiologists continuing to perform final reads and clinical validation.

Importantly, this workflow introduces a dual-layer interpretive process: an initial algorithmic assessment for triage and prioritization, followed by the radiologist's definitive clinical interpretation and diagnostic reporting. Both steps represent separate yet distinct contributions to the diagnostic process and require governance, validation, and integration into clinical operations.

Such workflows effectively create a dual-review paradigm – an initial AI-assisted prioritization or preliminary assessment followed by clinician interpretation – yet current payment models do not recognize the clinical value, operational efficiency, or quality improvement associated with AI-enabled workflow optimization. At this time, the absence of a reimbursement mechanism or established payment pathway for providers that have adopted this AI-driven triage model results in providers bearing all of the associated costs.

HHS has an opportunity to advance its stated goals of reducing administrative burden, accelerating responsible innovation, and improving patient access to timely, high-quality care by providing clear, flexible policy guardrails for AI-enabled clinical workflows. Thoughtful clarification regarding the regulatory and reimbursement treatment of tools including AI-assisted triage and clinician interpretation would promote adoption of technologies that demonstrably increase efficiency, support clinician decision-making, and improve patient outcomes, while avoiding prescriptive requirements that could slow innovation or impose unnecessary compliance costs on providers. **We encourage the Department to pursue a risk-based, outcomes-focused approach with reimbursement that enables providers to integrate AI tools into clinical practice in a manner that is scalable, interoperable, and aligned with patient safety and quality objectives.**

Supporting Provider-led AI Innovation Efforts

CHIME member Baptist Health Jacksonville provides a compelling example of how health systems can innovate effectively and efficiently to save the lives of some of the most vulnerable patient populations they care for. Child sex trafficking is a hidden epidemic, with an estimated 1 in 7 endangered runaways in the U.S. being likely victims.³ In 2024 alone, the National Center for Missing & Exploited Children received more than 27,800 reports of possible child sex trafficking.³ Research indicates that nearly 88% of trafficking victims interact with healthcare providers while being exploited, making healthcare providers uniquely situated to identify potential trafficking cases and intervene.⁴ The organization developed and embedded a pediatric emergency care screening protocol in its EHR to systematically detect potential child sex trafficking. This protocol integrates automated risk alerts, trauma-informed clinician training,

³ National Center for Missing & Exploited Children. Child sex trafficking. MissingKids.org. Published 2025. Accessed September 2, 2025. <https://www.missingkids.org/theissues/trafficking>

⁴ Lederer LJ, Wetzel CA. The health consequences of sex trafficking and their implications for identifying victims in healthcare facilities. *Annals Health L.* 2014;23:61.

and streamlined reporting workflows. Over one year of implementation, the program achieved high screening compliance and identified numerous at-risk children, and the tool has since been incorporated into Epic's EHR to enable broader dissemination to other health systems using the platform.

There is a need for system-level adoption of such tools and policy measures, including funding and training initiatives, to maximize healthcare organizations' (HCOs) impact in the fight against the significant public health crisis⁵ of child sex trafficking. Yet emergency departments (EDs), already stretched thin, often miss opportunities to detect and aid trafficked children. The experience at Baptist Health demonstrates that integrating specialized trafficking screenings into EHR workflows can help close this gap. However, to use the screening tool successfully, organizations must also train clinicians on how to sensitively screen patients and what to do when a screening is positive.

Further research is needed to determine the screening tool's accuracy and refine it to reduce false positives and to improve its identification rate for younger children and non-English speakers. Survivors of trafficking and frontline healthcare staff should be engaged in the design and continuous improvement of screening tools to ensure that they are practical, ethical, and effective.

To extend these successes, HHS should consider dedicated funding to help EDs implement trafficking screening programs, hire specialized personnel, and provide ongoing staff education. Similarly, the Centers for Medicare & Medicaid Services (CMS) could reimburse screening and intervention efforts as part of routine care. Health systems of all sizes have a role to play, not only in treating the medical consequences of trafficking, but in helping to prevent it, starting from the moment a vulnerable child comes through their doors.

HHS and CMS are well-positioned to support these and other innovative and effective efforts through targeted funding, guidance, and reimbursement frameworks, thereby enabling health systems to maximize their impact in addressing this and other significant public health crises. This is just one example of the critical work our members are doing in their communities that illustrates how – with modest resources – health systems can create impactful, evidence-informed interventions that protect the most vulnerable patients and serve as a model for replication nationwide.

Members have observed that as trust and reliance on AI tools increase, associated costs are rising substantially. For example, one member noted that an AI imaging tool in use since 2019 has experienced a 110 percent cost increase, highlighting the potential financial burden on HDOs and underscoring the need for consideration of cost impacts in AI adoption and reimbursement policy.

CHIME encourages HHS to expand the range of incentives available to promote the use of AI in clinical care, as such incentives are essential to accelerating adoption. Additionally, CMS should consider implementing enhanced reimbursement mechanisms or bonus payments for healthcare providers who deploy AI in clinical care, recognizing the operational costs, efficiency gains, and measurable improvements in patient care that such technologies can deliver.

⁵ Land A, Kino L. Florida ranks third in the number of reported human trafficking cases nationwide. WUFT News. Published February 6, 2024. Accessed September 2, 2025. <https://www.wuft.org/public-safety/2024-02-06/florida-ranks-third-in-the-number-of-reported-human-trafficking-cases-nationwide>

CHIME has and continues to be a staunch champion when it comes to the need for the use of innovation and digital health solutions – including AI – aimed at facilitating better patient care. However, it is imperative that providers across the full continuum of care – including acute, ambulatory, post-acute, behavioral health, individual providers, and safety-net settings – are deliberately and equally included in any AI framework’s design, implementation, and operational support strategies. Further, Medicare spending for beneficiaries receiving post-acute care (PAC) services is substantial and represents a significant opportunity for innovation. Given the considerable resources CMS devotes to beneficiaries in long-term and post-acute care (LTPAC) settings, targeted policy levers, aligned financial incentives, and dedicated technical assistance will be essential to ensure these providers are fully included as the health system transitions toward a more connected, interoperable, and patient-centered data environment.

Another way that regulatory burdens could be lessened is by updating the CMS Stark and Office of the Inspector General (OIG) Anti-Kickback Statute (AKS) policies to clarify the provisions involving cybersecurity donation policies. Larger providers continue to express reluctance to donate cyber technology to other lesser-resourced organizations because they fear they could incur liability if the smaller provider experiences a breach or cyber incident. These liability fears have deterred better resourced organizations from making these much-needed donations. Further, expanding the definition of what types of technology can be donated (e.g., AI tools) would also spur greater uptake.

Providers are increasingly turning to chatbots for care management to enhance patient engagement and streamline administrative tasks which may offer cost-effective care modalities. These AI-powered chatbots can assist patients with appointment scheduling, medication reminders, and answer frequently asked questions, thereby reducing the workload on healthcare staff. Additionally, chatbots can provide personalized health advice and monitor patient symptoms, ensuring timely interventions and improving overall care management. This innovative approach not only improves efficiency but also ensures that patients receive consistent and accessible support.

One member piloting an AI-enabled conversational system reported meaningful reductions in depression, loneliness, and feelings of social isolation among residents in a senior living community, describing the technology as a “game changer” for resident engagement. This experience was highlighted in [The Wall Street Journal in June 2025](#), which reported early evidence that AI-enabled conversational companions may alleviate loneliness and improve mental health among older adults in senior living settings.

Recent peer-reviewed research further supports these observations. A [feasibility study](#)⁶ published in the *Journal of the American Medical Directors Association* evaluated an AI-based, voice-driven telephone intervention designed to facilitate conversational engagement among older adults in post-acute and long-term care (PALTC) settings. Participants engaged with the system through scheduled and on-demand telephone interactions over a four-week period. “Preliminary analyses showed reductions in depressive and anxiety symptoms, especially among participants with higher baseline severity.”⁶

Loneliness, depression, and anxiety are prevalent among older adults in PALTC settings and are associated with adverse health outcomes, increased healthcare utilization, and diminished quality of life. Traditional psychosocial interventions often face workforce constraints and

⁶ Palace, Zachary J., et al. “Leveraging an AI-driven voice companion to enhance mental health in older adults in post-acute and long-term care settings: A feasibility study.” *Journal of the American Medical Directors Association*, vol. 27, no. 3, Mar. 2026, p. 106047, <https://doi.org/10.1016/j.jamda.2025.106047>.

challenges related to scalability and sustained engagement. AI-enabled conversational tools may serve as a scalable adjunct to existing care models, supporting social engagement and behavioral health monitoring while reducing strain on clinical and caregiving staff. Such tools may also support family caregivers by providing consistent social engagement and monitoring signals between visits, potentially reducing caregiver burden and improving communication with care teams. Additional research is needed, with the study noting that, “Future work should include larger, randomized controlled trials with extended follow-up to validate results and explore population-specific benefits.” However, the early findings suggest that AI-driven conversational technologies have significant potential to enhance patient-centered care and support healthy aging.

Tools such as AI-enabled conversational companions in PALTC settings illustrate the potential of non-diagnostic AI to address social isolation, support behavioral health, and augment limited workforce capacity. **HHS could further support pre- and post-deployment evaluation processes for non-medical AI tools that show promise through targeted federal funding and coordination mechanisms. Grants and cooperative agreements through the Department could support pragmatic clinical trials, implementation science studies, and standardized evaluation frameworks for these tools, including those used for conversational AI in aging and behavioral health contexts. Prize competitions could incentivize scalable and interoperable tools. These mechanisms would accelerate evidence generation while reducing duplicative testing burdens on providers and vendors.**

A coordinated federal approach to evaluation of non-medical AI tools – grounded in risk-based, outcomes-focused principles – could support responsible innovation, promote interoperability, and reduce burden while ensuring patient-centered adoption of AI-enabled technologies across the healthcare system.

Frameworks & Standards

As providers chart their AI course, they are looking for ways to deploy and use AI in an ethical manner. Many hospitals and healthcare systems have AI councils, acceptable use, and/or governance policies established within their organizations, though rural, safety-net, and smaller providers are either just getting started or have not yet taken the leap. Providers may also be served by having structures and processes to report potential safety events related to the AI tools. In the beginning, this could be an internal process. There are emerging AI frameworks and standards that serve as a good starting point for HDOs embarking on or refining their AI adoption journey. They are intended to enable responsible, ethical, and effective advancements in health AI on behalf of the patients and communities they serve. Resources exist to help providers navigate the AI journey; [CHIME's AI Principles](#) offer a detailed overview of existing standards and frameworks, and below we offer a high-level overview of just a few.

The [NIST AI Risk Management Framework \(AI RMF\)](#), which was developed in conjunction with the private sector, and “intended for voluntary use and to improve the ability to incorporate trustworthiness considerations into the design, development, use, and evaluation of AI products, services, and systems.” America’s AI Action Plan calls for revising the existing Framework. Providers are already deeply familiar with the NIST Cybersecurity Framework (CSF) and have voluntarily adopted it. Released in 2023, the NIST AI RMF offers a complementary framework to better manage risks to individuals, organizations, and society associated with AI. NIST is also developing a way to reach consensus faster around AI standards development – typically a lengthy process – through their “Zero Drafts” pilot project.

The [National Academy of Medicine's Artificial Intelligence Code of Conduct](#) is “aimed at providing a guiding framework to ensure that AI algorithms and their application in health, health care, and biomedical science perform accurately, safely, reliably, and ethically in the service of better health for all.”

The [Health AI Partnership \(HAIP\)](#) is a multi-stakeholder, provider-driven effort spearheaded by Duke University, and offers [tools](#) to help providers navigate the purchase and deployment of AI including designing and testing clinician workflow and disseminating information to end users. Their [Health Equity Across the AI Lifecycle \(HEAAL\) Framework](#) is community-informed framework developed through extensive collaboration with an interdisciplinary team of developers and inputs from seventy seven practitioners involved in various functions of HDOs. HEAAL is a comprehensive measure intended to be used whenever an HDO considers implementing an AI solution – and is empowering providers to safely and fairly use AI through the use of standards to focus on procurement, development, clinical integration, and lifecycle management.

Privacy, Security, & Third-Party Solutions

AI's integration into healthcare heightens the need for robust cybersecurity and privacy protections. Providers face increasing regulatory burdens, fragmented policies, and rising threats from AI-driven cyberattacks; therefore, the critical and inextricable domain of cybersecurity as it intersects with the clinical deployment of AI cannot and should not be ignored. [CHIME's AI Principles](#) recognize that trustworthy, scalable, and clinically responsible AI cannot exist apart from a resilient cybersecurity posture, given the uniquely targeted threat environment facing healthcare providers and the sensitivity, volume, and interoperability of clinical data assets.

We therefore urge the Department to evaluate clinical AI policy within the broader operational reality confronting hospitals and health systems – namely, a persistently hostile cyber landscape marked by ransomware, third-party supply chain vulnerabilities, and adversarial nation-state activity – while aligning federal expectations for AI governance with existing statutory and regulatory obligations under the Health Insurance Portability & Accountability Act (HIPAA), the Health Information Technology for Economic and Clinical Health Act (HITECH), and related frameworks. As further detailed in [our principles](#), cybersecurity must be treated not as an ancillary safeguard, but as a foundational prerequisite to safe, ethical, and sustainable clinical AI adoption.

Vendors Routinely Shift More Risk to Providers

HIPAA privacy protections extend only to HIPAA covered entities (CEs) which include providers, payers and healthcare clearinghouses, as well as any business associates (BAs) of these entities. Companies that have access to PHI and do business with a HIPAA CE are required by law to safeguard the data. However, some vendors try to avoid risk by refusing to agree to certain provider terms and provisions in BAAs, shifting more risk to providers. As providers ink agreements with AI vendors, they have reported vendors attempting to shift liability by refusing to accept key terms in the BAA or overriding restrictions in the BAA – exposing them to additional risk.

Our members also report vendors using contracts that append the BAA as an attachment and use language in the main contract that overrides all other language as it pertains to data use.

Some AI vendors have refused to sign BAAs with providers, while others have elected to train their models using de-identified data instead. Although HIPAA requires a standard BAA, the regulatory language sets only a minimum threshold—stating that a BA may use or disclose PHI solely as permitted by the agreement with the CE. The challenge arises when vendors embed broad data use rights into the main service agreements, effectively circumventing the intent of the BAA. Many vendors assert that de-identified data falls outside HIPAA's scope, using this claim to justify expansive data use, often in ways that make providers uncomfortable. However, de-identification is highly subjective and context-dependent, and vendor language—such as assurances that “de-identified data will not be used in any way that could identify a natural human”—raises serious concerns about enforceability and the potential for re-identification. As HHS’ OCR [has emphasized](#), the Privacy Rule recognizes two permissible de-identification methodologies: (1) a formal determination by a qualified expert, or (2) the removal of specified identifiers coupled with the absence of actual knowledge that the remaining information could be used to identify an individual. Importantly, even when these methodologies are properly applied, the resulting data retains a residual risk of identification; while the probability is very small, it is not zero, and de-identified data may, in certain circumstances, be linked back to the individual to whom it pertains.

In a perfect world, the original intent of HIPAA would be reinforced: data should be used solely for the purposes outlined in the agreement and only in ways that directly support the services for which the provider has contracted. **Unfortunately, many providers cannot afford to walk away from certain vendors, placing them in an impossible bind—underscoring the urgent need for increased oversight by the Office for Civil Rights (OCR) to ensure that the spirit of HIPAA is upheld.**

Healthcare providers are increasingly constrained when purchasing and procuring applications, medical devices, AI solutions, and other digital technologies. Vendors often deliver products with insufficient baseline security assurances, forcing providers to shoulder the burden of conducting individual risk assessments that are costly, duplicative, and resource-intensive. This fragmented approach undermines efficiency, slows adoption of innovative tools, and can leave critical vulnerabilities unaddressed across the ecosystem.

Third-Party Risk and Burden for Providers

A marketplace should be established for certified apps – including those leveraging AI – which have demonstrated their ability to meet a baseline set of security requirements. Establishing consistent, transparent, and enforceable security standards for third-party technologies is essential to reducing provider burden, strengthening supply chain resilience, and protecting patients from escalating cyber threats.

Providers continue to grapple with substantial vulnerabilities inherent in third-party solutions, underscoring the need for remedial action to safeguard the confidentiality, integrity, and availability of Americans’ health data. Providers need confidence that the technologies they purchase are secure; however, in the absence of a uniform set of standards that must be met, our members are forced to conduct their own risk assessments when procuring essential medical devices, apps, and AI solutions.

A rigorous review of third-party devices and applications is needed to ensure compliance and security best practices. Providers are saddled with these time-consuming and costly processes as they conduct their own assessments on the same technology being purchased by thousands of other providers, which is inefficient, redundant, and a waste of precious healthcare resources.

When delivering updates to software that deliver new AI capabilities or updates or changes AI components, vendors should be required to disclose those AI components, the type of AI and the data used, as well as disclose their related security assessment including addressing relevant threats and vulnerabilities. CHIME firmly believes that shifting risk and burden onto providers is slowing AI adoption in clinical care.

Conclusion

In closing, CHIME appreciates the opportunity to provide input in response to this RFI and to contribute to HHS-wide policy development concerning the clinical use of AI. On behalf of our members, we offer practical, provider-informed recommendations grounded in real-world implementation experience.

Our comments are intended to assist the Department in determining how its regulatory, reimbursement, and research and development authorities can be most effectively calibrated to support the responsible and timely adoption of AI in clinical care; foster public trust and confidence in modern health technologies; reduce regulatory and liability uncertainty that may impede innovation; and better align federal incentives so that AI deployment enhances productivity, alleviates administrative burden, lowers healthcare costs, and improves health outcomes for patients, caregivers, and communities.

To realize AI's full potential, providers must be equipped with the knowledge, resources, technical infrastructure, and policy certainty necessary to overcome adoption barriers. CHIME and its members remain steadfast in their commitment to leveraging technology to deliver high-quality, patient-centered care and to advancing interoperability and secure, appropriate access to health information across the care continuum. At the same time, we respectfully urge HHS to ensure that future rulemaking or guidance does not inadvertently impose duplicative or prescriptive requirements that could divert limited resources away from patient care, investments in digital technology such as AI tools, or cybersecurity investments.

CHIME and our members look forward to continuing to be a trusted stakeholder to the Department, and stand ready to serve as an ongoing resource as HHS advances its AI policy objectives. Collaborative engagement between the public and private sectors will be essential to achieving our shared goals and ensuring that AI strengthens – rather than complicates – the delivery of healthcare in the United States.

Should you have any questions or if we can be of assistance, please contact Chelsea Arnone, Senior Director, Federal Affairs at carnone@chimecentral.org.

Sincerely,

A handwritten signature in black ink, reading "Russell P. Branzell". The signature is fluid and cursive, with the first name "Russell" and last name "Branzell" clearly legible.

Russell P. Branzell, CHCIO, LCHIME
President and CEO
CHIME